BIOSIMILAR READINESS · ASSESSMENT

The Biosimilar Opportunity Your Plan Is Missing.

A six-factor framework for evaluating whether your plan is capturing biosimilar savings, or leaving them on the table because the PBM's formulary serves rebate revenue better than it serves your bottom line.

01 Before You Score: Data to Pull from Your PBM

Request these items by therapeutic category for every drug class with an approved biosimilar in the U.S. market. The third item, net cost comparison, is the single most diagnostic data request in the entire assessment. If your PBM resists providing it, that resistance is itself a finding.

- Current formulary placement for each biosimilar vs. reference brand
- Net cost comparison (after rebates) for each therapeutic category
- Plan document language addressing biosimilar substitution
- Member cost-sharing tier for biosimilar vs. reference brand
- Biosimilar dispensing rate by drug, last 12 months
- Provider outreach activity from PBM on biosimilar adoption

02 Score Your Readiness

FACTOR	STRONG	MODERATE	WEAK	SCORE
Formulary positioning	STRONG Biosimilar preferred tier with lower member cost-sharing than brand.	MODERATE Biosimilar same tier as brand. No cost-sharing incentive.	WEAK Biosimilar at higher tier, or not on formulary at all.	○ ○ ○
Auto-substitution policy	STRONG Plan document explicitly addresses biosimilar substitution aligned with state law.	MODERATE State law allows interchangeable substitution; plan document silent.	WEAK No plan document language; pharmacist substitution unclear.	○ ○ ○
Rebate transparency	STRONG Net cost comparison (after rebates) provided per category, on request or quarterly.	MODERATE Aggregate rebate reporting; no net cost comparison.	WEAK No net cost comparison available; rebate detail opaque.	○ ○ ○
Provider education	STRONG	MODERATE	WEAK	○ ○ ○

FACTOR	STRONG	MODERATE	WEAK	SCORE
	PBM actively communicates with prescribers; clinical evidence summaries provided.	PBM offers provider outreach but not consistently activated for your plan.	No provider education program; prescriber awareness left to chance.	
Member communication	STRONG Member communications address biosimilars by name; cost-sharing incentive explained.	MODERATE General member communications; biosimilar topic surfaced reactively.	WEAK Members first hear about biosimilars at the pharmacy counter.	○ ○ ○
Contract language	STRONG PBM contract includes biosimilar dispensing-rate guarantees with remediation.	MODERATE Contract references biosimilars but lacks dispensing or net-cost guarantees.	WEAK Contract silent on biosimilars. PBM has no obligation to prioritize them.	○ ○ ○

Illustrative scoring framework for educational purposes. Actual readiness varies by plan. Three or more Weak scores typically signals meaningful unrealized biosimilar savings. Factor 3 (rebate transparency) is the single most diagnostic data point: if you do one thing, request the net cost comparison.

03 Categories with Approved Biosimilars to Audit

For each therapeutic category below, request the net cost comparison from your PBM and verify formulary placement. These are the categories where biosimilar savings opportunities most often go unrealized.

Adalimumab biosimilars	Reference: Humira	*Multiple biosimilars approved. The largest savings opportunity in most plans; check formulary tier and net cost first.*
Infliximab biosimilars	Reference: Remicade	*Long-established biosimilars. Typically administered via medical benefit; site-of-care also matters.*
Rituximab biosimilars	Reference: Rituxan	*Oncology and rheumatology indications. Net cost comparison varies meaningfully by plan rebate structure.*
Trastuzumab / Bevacizumab biosimilars	Reference: Herceptin / Avastin	*Oncology. Site-of-care, channel routing, and clinical pathway alignment all influence realized savings.*
Pegfilgrastim biosimilars	Reference: Neulasta	*Supportive oncology care. Established biosimilar competition; high adoption is achievable when formulary aligns.*

04 Optimization Actions by Factor

IF WEAK ON FACTOR 1 · FORMULARY POSITIONING

Move biosimilars to preferred tier with lower cost-sharing.

Tier placement is the most direct lever. Request a formulary update for every category where a biosimilar is available, with the biosimilar at preferred status and member cost-sharing materially below the brand. Expect rebate-related pushback; the net cost comparison from Factor 3 is your counter-argument.

IF WEAK ON FACTOR 3 · REBATE TRANSPARENCY

Request the net cost comparison this quarter.

For every category with a biosimilar, request a written net-cost-after-all-rebates comparison: brand vs. biosimilar. If the PBM resists or only offers aggregate rebate metrics, escalate. The single data request that changes the entire conversation.

IF WEAK ON FACTORS 4 OR 5 · EDUCATION / COMMUNICATION

Activate provider and member outreach before the next renewal.

Most PBMs have provider outreach programs and member communication templates available; activation is on the plan sponsor to request. Build a communication plan with prescriber-targeted clinical evidence and member-targeted cost-sharing benefit messaging. Time it to formulary changes.

IF WEAK ON FACTOR 6 · CONTRACT LANGUAGE

Add biosimilar-specific provisions at next renewal.

Biosimilar dispensing-rate guarantees, brand-to-biosimilar transition timelines, net cost optimization requirements, and remediation if adoption targets miss. Of approximately 100 contracts we review annually, biosimilar-specific language remains uncommon. The renewal redline is where the gap closes.